

FINAL PROJECT – INDIVIDUAL CASE SAFETY REPORT (ICSR)

Complete Pharmacovigilance Case Assessment

Task ID: task5

Case basis: The same simulated amoxicillin case used in the preceding ADR reporting and causality-assessment tasks.

1. PATIENT IDENTIFICATION

Patient Identifier	Patient A (de-identified)
Age	45 years
Sex	Female
Patient Status	Adult
Relevant History	Not provided in the case information

2. SUSPECTED MEDICINE

Suspected Drug	Amoxicillin
Indication	Not documented
Dose	Not documented
Route	Not documented
Therapy Start	Approximately 5 days before the report; exact date not provided
Therapy Duration at Onset	Approximately 3 days before reaction onset

3. ADVERSE DRUG REACTION

Reaction	Skin rash with itching; nausea and dizziness were also reported in the earlier ADR case documentation.
Onset	Approximately 3 days after starting amoxicillin
Clinical Description	The principal documented reaction for this final assessment is a skin rash with itching. The available case does not provide detailed morphology, distribution, laboratory findings, or examination findings.
Seriousness	Non-serious
Outcome	Recovered after stopping the suspected drug

4. CONCOMITANT MEDICATION / OTHER INFORMATION

Concomitant Medication	Paracetamol, used as needed (PRN)
Alternative Causes	No alternative cause is documented in the supplied case.
Medical History	Not provided
Allergy History	Not provided
Laboratory / Diagnostic Data	Not provided

5. ACTION TAKEN WITH SUSPECTED DRUG

Action	Amoxicillin was stopped / discontinued.
Dechallenge	Positive: the patient recovered after discontinuation.
Rechallenge	Not performed / not documented.
Treatment for ADR	Not documented.

6. CAUSALITY ASSESSMENT

Assessment Method	Clinical/standard pharmacovigilance causality assessment
Temporal Relationship	Positive: the reaction began about 3 days after amoxicillin was initiated.
Dechallenge	Positive: recovery occurred after stopping amoxicillin.
Alternative Causes	None identified from the available information.
Rechallenge	Not available.
Final Causality	Probable / Likely
Assessment Rationale	The timing of onset is compatible with a drug-related reaction, the patient improved after withdrawal, and no alternative cause is documented. Rechallenge evidence is absent, so a certain classification is not assigned.

7. SERIOUSNESS & EXPECTEDNESS CLASSIFICATION

Seriousness	Non-serious
Reason	No death, life-threatening event, hospitalization, significant disability, congenital anomaly, or other medically important serious outcome is documented.
Expectedness	Expected
Reason	Skin rash and hypersensitivity-type reactions are recognized adverse effects associated with amoxicillin.

8. ICSR SUMMARY

ICSR Element	Final Information
Identifiable patient	Yes – adult female, 45 years, de-identified as Patient A.
Suspected medicinal product	Amoxicillin.
Adverse event	Skin rash and itching; nausea and dizziness also documented in the earlier case report.
Temporal relationship	Reaction began approximately 3 days after drug initiation.
Dechallenge	Positive – recovered after stopping the drug.
Rechallenge	Not performed / not documented.
Seriousness	Non-serious.
Expectedness	Expected.
Causality	Probable / Likely.

9. DRUG SAFETY ASSOCIATE ASSESSMENT

The case represents a valid simulated pharmacovigilance safety report because an identifiable patient, a suspected medicinal product, and a suspected adverse event are available. The available evidence supports a probable association between amoxicillin and the reported reaction. The event is classified as non-serious and expected. Important missing information includes exact dose, route, treatment dates, indication, medical/allergy history, detailed clinical description, treatment provided, and exact recovery date. These data should be requested during follow-up where possible.

10. FINAL CONCLUSION

Final case classification: Amoxicillin-associated skin rash/itching – **Non-serious, Expected, Probable/Likely.**

The positive temporal relationship and positive dechallenge support the suspected drug relationship. No rechallenge was performed, and no alternative cause is documented.

Training note: This ICSR is a simulated educational document prepared from the case information used in the preceding tasks. It is not a real patient report and should not be submitted to a regulatory authority as an actual safety case.